

Client: Marcus Reyes
DOB: 04/11/1991
Provider: Adrian Dai

Appointment: Individual appointment on May 4, 2026
1:30 pm - 2:20 pm CT, 50 min
Billing code: 90834 - Psychotherapy, 45 min

[BPS] Intake Assessment

1. Presenting Problem:

"I fell at home and my brother brought me to the ED. They said I was withdrawing."

2. Signs and Symptoms (DSM-V-TR based) resulting in impairment(s):

(Include current examples for treatment planning, e.g., social, occupational, affective, cognitive, physical)

Patient meets DSM-5-TR criteria for Alcohol Use Disorder, severe (≥ 6 criteria endorsed: tolerance, withdrawal, hazardous use, continued use despite social/occupational consequences, time spent obtaining/using, unsuccessful cut-down attempts); Sedative-Hypnotic Use Disorder, severe (alprazolam); and Major Depressive Disorder, recurrent, moderate (anhedonia, depressed mood, sleep disturbance, fatigue, appetite change, low self-worth, poor concentration, psychomotor change — PHQ-9 = 18, moderately severe). Current alcohol withdrawal syndrome with tremor, sweats, anxiety, insomnia (CIWA-Ar = 12). Resulting impairment spans: occupational (job loss 6 wks ago for intoxication at work), social (isolation from former sober peers), affective (severe anxiety GAD-7 = 15, depression PHQ-9 = 18), physical (witnessed fall, uncontrolled HTN, alcohol-related hepatic injury), and cognitive (poor concentration, impaired insight/judgment on MSE).

3. History of Presenting Problem

(Events, precipitating factors, or incidents leading to need for services):

34yo male with 4-year history of heavy alcohol use and 18-month history of daily alprazolam (originally prescribed for anxiety, now obtained illicitly), presenting 36 hours after last drink with tremor, sweats, sleep <2h, and a witnessed fall at home. Referred from ED for medically monitored withdrawal management after CIWA-Ar of 12 on arrival. Lost employment 6 weeks prior due to intoxication at work; DUI charge pending. Ambivalent about treatment ("I'm here because my brother said so").

Frequency/duration/severity/cycling of symptoms:

Alcohol: 4 years of escalating daily use, currently ~14 standard drinks/day. Alprazolam: 18 months of daily use, escalated from a legitimate 0.5 mg PRN prescription to illicit 1 mg daily. Cannabis: recreational, 2–3x weekly. Acute alcohol withdrawal currently moderate (CIWA-Ar = 12). Benzodiazepine withdrawal anticipated but not yet manifest. Depressive and anxious symptoms have been continuously present x 4 years with worsening since job loss 6 weeks ago.

Was there a clear time when Sx worsened?

Yes — substance use and depressive symptoms escalated sharply 6 weeks ago following job loss (terminated for intoxication at work). Acute withdrawal began approximately 36 hours prior to admission after a forced cut-down attempt (no alcohol available at home over the weekend).

Family mental health history:

Father with alcohol use disorder; mother with depression

4. Current Family and Significant Relationships

Strengths/support:: Brother Daniel willing to engage; provided transportation to ED.

Stressors/problems:: Brother is an active substance user (alcohol, cannabis). Patient is housing-dependent on the brother. No partner, no children. Parents live out of state; minimal contact.

Recent changes:: Job loss 6 weeks ago; pending DUI charge. Brother delivered ultimatum about housing.

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Changes desired:: Stable sober housing; restored autonomy; reconnect with mother.

Comment on family circumstances:: Father with untreated alcohol use disorder. Mother with treated major depression. Patient is the younger of two siblings.

5. Childhood/Adolescent History

(Developmental milestones, past behavioral concerns, environment, abuse, school, social, mental health)

Normative early development. Raised in two-parent household until parental separation at age 11. First alcohol use at age 16 (family setting, moderate). First cannabis at age 17 (peer setting). High school graduate; completed 18 credits at community college. No documented abuse or neglect. 2019 MVA at age 28 with mild TBI — brief LOC, evaluated and discharged from ED. No documented cognitive sequelae, but patient reports lingering difficulty with sustained concentration since.

6. Social Relationships

Strengths/support:: Brother Daniel (limited — actively using). Former AA sponsor from 2023 cycle, lost contact.

Stressors/problems:: Social isolation post-job-loss. Drift from former sober peer group. No current peer-recovery network.

Recent changes:: Has not contacted former sponsor in 12+ months. Sober friends drifted as drinking escalated in 2024.

Changes desired:: Reconnect with former sponsor; build new sober peer network through structured group.

7. Cultural/Ethnic

Strengths/support:: Identifies as Hispanic/Latino. Strong cultural value placed on family ties (familismo).

Stressors/problems:: Cultural shame around mental health and substance use. Internalized 'be a man, handle it yourself' narrative.

Beliefs/practices to incorporate into therapy:: Family-systems framing welcomed. Spanish-language psychoeducation materials acceptable. Open to culturally-aware peer matching.

8. Spiritual/Religious

Strengths/support:: Raised Catholic. Faith was a source of comfort in childhood; mother and grandmother were practicing.

Stressors/problems:: Currently disconnected from practice. Reports guilt and shame around substance use as a barrier to returning to church.

Beliefs/practices to incorporate into therapy:: Open to facility chaplaincy if available. Open to AA/NA frameworks that incorporate higher-power language without requiring it.

Recent changes:: Has not attended Mass in 4+ years. Grandmother (primary spiritual influence) died in 2022.

Changes desired:: Wants to 'find some peace' but not actively pursuing a specific practice at this time.

9. Legal

History:: No prior arrests or convictions.

Status/impact/stressors:: Pending DUI charge (2026-04-12). Court date TBD; assigned public defender. May lose driver's license. Stressor weighing heavily on treatment ambivalence and willingness to engage.

10. Education

Strengths:: High school graduate. Completed 18 credits of community college coursework.

Weaknesses:: Did not complete associate's degree. Reports difficulty with sustained concentration since 2019 mTBI, exacerbated by current substance use.

11. Employment/Vocational

Strengths/support:: 5-year tenure in warehouse logistics (2020–2026). Considered reliable when sober.

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Stressors/problems:: Unemployed x 6 weeks. Terminated for intoxication at work (workplace breathalyzer policy violation). Income loss is a major stressor. No unemployment income — termination was 'for cause.' Identity disruption: "I'm not used to not working."

12. Military

Current impact:: No military service history.

13. Leisure/Recreational

Strengths/support:: Previously enjoyed watching basketball and walking in the neighborhood.

Recent changes:: Has not engaged in leisure activities x 6 months — substance use has crowded everything else out.

Changes desired:: Wants to 'get back into walking' as a wellness practice during/after treatment.

14. Physical Health

Summary of health:: Active: uncontrolled essential hypertension, suspected alcoholic hepatitis (not biopsy-confirmed), remote mild TBI (2019). No current primary care relationship. Last PCP visit 2024 for naltrexone prescription.

Physical factors affecting mental condition:: Severe sleep deprivation (<2 h/night for several days pre-admission). Withdrawal physiology (autonomic arousal, tremor) directly affecting cognition, mood, and concentration. Hepatic dysfunction may affect medication metabolism — clinical relevance for benzo taper and any future SSRI/SNRI selection.

Vitals (BP, HR, RR, temp, SpO2):: BP 158/96; HR 104; RR 18; T 99.1°F; SpO2 97%.

Current medications (name, dose, frequency, prescriber):: None at admission. Was prescribed lisinopril 10 mg PO daily by PCP in 2024 for HTN; non-adherent x 18 months. To be re-initiated on admission day 4 per inpatient medical orders.

Allergies:: No known drug allergies (NKDA).

Recent labs (date, panel, results):: Hepatic panel 2026-05-04: AST 142 U/L (ref 10–40), ALT 98 U/L (ref 7–56), GGT 312 U/L (ref 9–48), total bilirubin 1.4 mg/dL (ref 0.1–1.2) — consistent with alcohol-related hepatic injury. UDS 2026-05-04: POSITIVE for ethanol, benzodiazepines, cannabinoids; NEGATIVE for opioids, cocaine, amphetamines.

15. Chemical Use History

Summary of use:: Polysubstance: heavy daily alcohol (4 years), daily illicit alprazolam (18 months), recreational cannabis (years). Denies opioid, stimulant, and hallucinogen use; UDS corroborates. Active alcohol withdrawal at intake (CIWA-Ar = 12); benzodiazepine withdrawal anticipated.

Patient's perception of problem:: Minimizes severity: "I drink a lot but it's not that bad." Attributes ED visit to "just a fall, not the drinking." Acknowledges alprazolam use is "getting out of hand" but does not connect it to the withdrawal-anticipation finding. Ambivalent about treatment: "I'm here because my brother said so."

Per-substance breakdown (alcohol, sedative-hypnotics, cannabis, opioids, stimulants, other) — for each:

first/last use, route, frequency, amounts, withdrawal hx, prior tx:: • Alcohol: first use age 16; pattern ~14 standard drinks/day x 4 years (vodka, beer); last use 2026-05-03; route oral; withdrawal history — Multiple episodes; one prior ED visit for seizure precaution; prior treatment — 2023 outpatient, 2024 naltrexone trial.

• Alprazolam (Xanax): first use age 32; pattern 1 mg PO daily x 18 months (originally Rx, now illicit); last use 2026-05-03; route oral; withdrawal history — Anticipated; not yet manifest at intake; prior treatment — None.

• Cannabis: first use age 17; pattern 2-3 joints per week, recreational; last use 2026-04-28; route inhaled; withdrawal history — None reported; prior treatment — None.

• Opioids: DENIES current or past opioid use. COWS recorded as N/A; documented to demonstrate negative-finding handling

AUDIT-C (date, items, total, interpretation):: AUDIT-C administered 2026-05-04: frequency 4, typical quantity 4, heavy episodic 3 — total 11 (positive, high risk).

DAST-10 (date, items, total, interpretation):: DAST-10 administered 2026-05-04: total 7 (substantial level, indicates intensive assessment and treatment indicated).

16. Counseling/Prior Treatment History

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Summary of prior treatment:: 2023 — Outpatient individual counseling, 6 weeks at community mental health center. Self-discontinued. 2024 — PCP-prescribed naltrexone 50 mg PO daily x 3 months; non-adherent and discontinued. No prior residential or inpatient SUD treatment.

Benefits of previous treatment:: Brief reduction in drinking (~30% by self-report) during the 2023 counseling cycle.

Setbacks of previous treatment:: Self-discontinued before completing planned course in 2023. Naltrexone non-adherence with no follow-up. No prior MAT, no prior recovery group attendance.

17. Past Psychiatric History

Prior psychiatric diagnoses (DSM-5-TR / ICD-10):: • Major Depressive Disorder, recurrent, moderate (ICD-10 F33.1; DSM-5-TR 296.32; onset 2019)

• Generalized Anxiety Disorder (ICD-10 F41.1; DSM-5-TR 300.02; onset 2020)

Psychiatric hospitalizations (year, facility, reason, duration):: • 2021 — Memorial Behavioral Health — Suicidal ideation, voluntary 72-hour hold

Prior psychiatric medications (drug, dose, dates, prescriber, response):: • Sertraline 100mg — 2020-2022 — Self-discontinued

• Alprazolam 0.5mg PRN — 2023 — Escalated to daily then illicit

Self-harm history:: One episode of superficial cutting age 22; no attempts; no recent ideation with plan

Prior suicide attempts (lifetime):: None.

18. Mental Status Exam

Appearance:: Disheveled, appropriate for age, malodorous of alcohol on arrival

Behavior / psychomotor:: Cooperative, mild psychomotor restlessness

Speech:: Normal rate, mildly pressured

Mood (client-reported):: Depressed and anxious

Affect (clinician-observed):: Constricted, congruent with mood

Thought process:: Linear and goal-directed

Thought content:: Passive death wish; no active SI/HI; no delusions

Perception:: No hallucinations

Cognition / orientation:: Alert and oriented x3; MoCA deferred due to withdrawal

Insight:: Limited — minimizes severity of substance use

Judgment:: Impaired — recent fall, continued use despite consequences

Standardized screening — PHQ-9 (date, items, total):: PHQ-9 administered 2026-05-04: anhedonia 3, depressed mood 3, sleep 3, fatigue 2, appetite 2, self-worth 2, concentration 2, psychomotor 1, suicidal ideation 0 — total 18 (moderately severe).

Standardized screening — GAD-7 (date, items, total):: GAD-7 administered 2026-05-04: nervousness 3, uncontrollable worry 3, excessive worry 2, trouble relaxing 2, restlessness 2, irritability 2, fear 1 — total 15 (severe).

19. Risk Assessment

Suicide risk — C-SSRS (date, items, interpretation):: C-SSRS administered 2026-05-04: Q1 no, Q2 YES ("I sometimes wish I weren't here"), Q3 no, Q4 no, Q5 no, Q6 no lifetime suicidal behavior. Risk: low-intensity passive ideation; no active SI/plan/intent.

Homicidal ideation:: Denies. No ideation, plan, intent, or lifetime history.

Withdrawal risk — CIWA-Ar (date, items, total):: CIWA-Ar administered 2026-05-04 14:30: nausea/vomiting 2, tremor 3, sweats 2, anxiety 2, agitation 1, tactile 0, auditory 0, visual 0, headache 1, orientation 1 — total 12 (moderate).

Withdrawal risk — COWS (date, items, total, or N/A):: COWS: not administered. Patient denies any opioid use; UDS negative for opioids. Documented for completeness.

Violence / aggression risk:: Low. No history of interpersonal violence; no current threats or ideation.

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Overall risk level:: Moderate — driven by withdrawal severity (CIWA-Ar = 12), passive suicidal ideation (C-SSRS Q2-positive), and unstable recovery environment.

Safety plan / precautions:: Inpatient monitoring with CIWA-Ar q4h; nursing line of sight if CIWA $\geq$ 15. Fall precautions in place. Suicide precautions: standard observation given passive ideation without plan/intent. Re-administer C-SSRS prior to any level-of-care change per NPSG.15.01.01.

20. Diagnosis

Primary diagnosis (DSM-5-TR / ICD-10):: Alcohol Use Disorder, severe, with withdrawal (F10.232; DSM-5-TR 303.90).

Secondary diagnosis (DSM-5-TR / ICD-10):: Sedative, Hypnotic, or Anxiolytic Use Disorder, severe (F13.20; DSM-5-TR 304.10).

Tertiary diagnosis (DSM-5-TR / ICD-10):: Major Depressive Disorder, recurrent episode, moderate (F33.1; DSM-5-TR 296.32).

Differential / rule-outs:: Generalized Anxiety Disorder (F41.1) is a co-occurring consideration, but anxiety symptoms may be substantially driven by current withdrawal physiology — reassess 2 weeks post-stabilization. Recommended level of care on admission: ASAM 3.7 — Medically Monitored Intensive Inpatient, driven by Dim 1 (active polysubstance withdrawal) and Dim 5 (unsafe recovery environment).

21. Treatment Plan

Goal 1 (domain, goal, target date, measure):: Domain: Substance use. Goal: Achieve and maintain 30 days of abstinence from alcohol and benzodiazepines. Target date: 2026-06-03. Measure: Daily breathalyzer, weekly UDS.

Goal 2 (domain, goal, target date, measure):: Domain: Mental health. Goal: Reduce PHQ-9 score from 18 to <10 by 60 days. Target date: 2026-07-03. Measure: Bi-weekly PHQ-9.

Goal 3 (domain, goal, target date, measure):: Domain: Recovery environment. Goal: Identify and engage one sober support person and one alternate housing option. Target date: 2026-05-25. Measure: Social-work documentation.

Interventions (list with frequency):: Cognitive Behavioral Therapy (CBT) — 3x/week individual

- Motivational Interviewing (MI) — woven across sessions
- Librium taper, gabapentin adjunct, thiamine
- Psychiatric medication evaluation for MDD/GAD (in week 2)
- Case-management for housing and DUI court navigation

Expected outcomes:: • Stable medical withdrawal within 7 days

- PHQ-9 < 10 by week 8
- Step-down to PHP/IOP by week 2 if Dims 1 and 2 resolve

Discharge / step-down criteria:: Step down to PHP (ASAM 2.5) when: (a) CIWA-Ar < 5 for 48 hours, (b) BP controlled on lisinopril, (c) sober housing option identified, (d) C-SSRS re-administered and stable. Target step-down date: 2026-05-15.